KENYA MEDICAL TRAINING COLLEGE

FACULTY OF CLINICAL MEDICINE

FINAL QUALIFYING EXAMINATIONS JULY 1995 OBS/GYNAE

PAPER II : M.C.Q.QUESTIONS

DATE 6/7/95

TIME: 3 HOURS

INSTRUCTIONS

* Write only your INDEX NUMBER on every sheet of paper used at the right hand corner.
* Do not write your names or College Numbers on answer sheets.
* All questions are compulsory.
* Answer by placing a X against the correct answer e.g.

TRUE	FALSE
___	___

* Do not guess as ½ mark will be subtracted for each wrong response given.
* Do not use a pencil.

		TRUE	FALSE
Q1.	Regarding the duration of pregnancy:-		
a.	Preterm labour occurs before the 38th week of pregnancy	X	
b.	Premature rapture of the membranes (PROM) occurs in the majority of cases of preterm labour	✓	
c.	The drugs salbutamol, vitodrine, ethanol are used to supress premature labour	X	X
d.	Postdatism occurs when pregnancy duration is greater than 42 weeks	X	
e.	L/S ratio is not used to determine foetal lung maturity		
Q2.	The commonest cause of vaginitis in adult women are:-		
a.	Trichomonas vaginalis	X	
b.	Candida albicans	X	
c.	Carcinoma of cervix		X
d.	Atrophic vaginitis		X
e.	All the above		X
Q3.	About Liquor amnii:-		
a.	Polyhydramnios is seen in anencephaly, spinabifida and atresia over the upper gastro-intestinal tract	X	
b.	Acute hydramnios is seen in multiple pregnancy		
c.	Oligochydrammios is found in fetal renal tract agenesis		
d.	Amniotic fluid contains creatinine	X	
e.	Amniotic fluid does not contain foetal urine		X

		TRUE	FALSE
Q4.	Regarding iso-immunization in pregnancy:-		
a.	Rhesus negative mothers are a minority in Kenya	X	
b.	All pregnant women should be tested to determine their rhesus group at the first prenatal visit	X	X
c.	Amnio centesis determines the severity of fetal haemolysis	X	
d.	All rhesus negative, unsensitized women who abort spontaneously should be given an injection of Rh-ant-D	X	X
e.	Direct Coomb's test is done on mothers blood		
Q5.	Surgical treatment of fibromyomata is indicated in mothers with:-		
a.	heavy or prolonged bleeding	X	
b.	Retention of urine	X	
c.	Tumours which obstruct labour	X	
d.	Tumours which have undergone torsion	X	
e.	All the above	X	
Q6.	Regarding infectious disease in pregnancy:-		
a.	Congenital rubella infection causes fetal malformations	X	
b.	Cytomegalovirus (CMV) infection causes foetal defects	X	
c.	Toxoplasmosis causes abortion	X	
d.	Syphillis in mother may lead to skeletal defects in the baby	X	
e.	Herpes simplex virus infections of the genital tract is unknown in Kenya		X
Q7.	Some complications of uterine fibroid include the following -		
a.	Menorrhagia	X	
b.	Infertility	X	
c.	Dysmenorrhoea		
d.	Retention of urine	X	
e.	All the above		
Q8.	About diabetes mellitus in pregnancy:-		
a.	(100m) or "Insulin" dependent diabetes mellitus appears in childhood or adolescence	X	X
b.	The cause of diabetes mellitus is not fully understood	X	
c.	Many cases of gestational diabetes respond to diet alone	X	X
d.	Pregnancy induced hypertention is a complication of diabetes	X	
e.	Oral glucose tolerance test (OGTT) is normal in pregnancy diabetes		X

		TRUE	FALSE
Q9.	The most appropriate treatment of chronic trichomonas vaginalis is:-		
	a. Tab Triberal 111 stat		X
	b. Tab Fasigyn iv (2000mg) stat		X
	c. Tab flagyl 400mg tds x 4	X	X
	d. All the above		X
	e. None of the above	X	X
Q10.	Regarding heart disease in pregnancy:-		
	a. Mitral stenosis is the most common disorder found	X	X
	b. Heart disease is rheumatic in origin in 70 per cent of cases	X	X
	c. Heart failure is a common complication in labour	X	
	d. Ergometrine is not given in the 3rd stage of labour	X	
	e. Assisted vacuum delivery is not necessary		X
Q11.	Cervical erosion can be caused by:-		
	a. Parturition	X	
	b. Contraceptive pill	X	
	c. Persistence of a condition which is normal in infancy	X	X
	d. None of the above		X
	e. All of the above	X	X
Q12.	Regarding anaemia in pregnancy:-		
	a. Anaemia in pregnancy is usually dimorphic in Kenya		
	b. Folic acid deficiency causes megaloblastic anaemia	X	
	c. Total dose of iron can be given using the compound imferon	X	
	d. Haemolytic anaemia in pregnancy can be due to sickle-cell anaemia	X	
	e. Anaemia in pregnancy does not cause foetal anaemia	X	
Q13.	The commonest cause of infertility in Kenya is/are:-		
	a. Polygamy		X
	b. Hormonal imbalance in women		X
	c. Tubal blockage	X	
	d. All the above		X
	e. None of the above		X
Q14.	Regarding hypertensive disease in pregnancy:-		
	a. Pre-eclampsia can be primary or secondary	X	
	b. Essential hypertension can cause pre-eclampsia	X	
	c. Eclampsia is accompanied by fits	X	
	d. Pregnancy induced hypertension cannot cause eclampsia		X
	e. In pre-eclampsia, cardiac output decreases		X

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		TRUE	FALSE
Q15.	The differential of eclampsia diagnosis include the following:-		
a.	Cerebral malaria	X	
b.	history of epilepsia	X	
c.	Meningitis	X	
d.	Brain abscess or tumour	X	
e.	None of the above		X
Q16.	Regarding antepartum haemorrhage:-		
a.	Placenta praevia is painless	X	
b.	Abraption placenta is painful	X	
c.	In placenta praevia the placenta is in the upper uterine segment		X
d.	Active treatment is the best in antepartum haemorrhage		
e.	There is no foetal mortality in abruptio placenta		X
Q17.	About the clinical course of labour:-		
a.	False labour does not cause dilation of the cervix	X	
b.	Braxton Hicks concentrations occurs in the prodromal stage of labour		X
c.	The first stage of labour has remarkable cervical dilation	X	
d.	Friedmann's Curve is a form of partogram		X
e.	Prolonged second stage of labour is normal in the primigravidae		X
Q18.	Causes of amenorrhoea include:-		
a.	Pregnancy	X	
b.	Psychological state	X	
c.	Hyperprolactinaemia	X	
d.	One of the above		X
e.	All the above	X	
Q19.	The most appropriate treatment of a 16 year old lady with irregular periods is:-		
a.	Observe her till it can regulate itself		X
b.	Dilation and Curratage		X
c.	Hormonal therapy in the form of pill	X	
d.	Antibiotic therapy is the best choice		X
e.	All the above		X
Q20.	About normal labour primigravida:-		
a.	Cervical dilation is about 1 cm per hour	X	
b.	Foetal head descent is important	X	
c.	Partogram is important in monitoring labour	X	
d.	Transverse lie causes obstruction of labour	X	
e.	Latent labour occurs during the second stage		X

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	TRUE	FALSE
Q21. Regarding antenatal care:-		
a. Antenatal care is about 50 years old		✓
b. Nutrition promotion is essential	X	
c. Antenatal screening for maternal disease is an important element in antenatal care	X	
d. Open neural tube detects in pregnancy are not easy to diagnose	✓	
e. Physical examination is unneccessary in antenatal care		X
Q22. Indications for Laparascopy include:-		
a. Diagnosis	X	
b. Infertility investigation	X	
c. Sterilisation	X	
d. All the above	X	
e. NOne of the above		X
Q23. About the diagnosis of pregnancy:-		
a. The uterus enlarges and softens in pregnancy	X	
b. The immunological tests for pregnancy is positive	X	
c. Ultrasound scan can visualize the foetus as early as 6 weeks	X	
d. Fetal heart sounds are positive signs of pregnancy	X	
e. Pseudocyesis is a form of pregnancy		✓
Q24. Contra-indications to insertion of I.U.C.D. is/are:-		
a. Lady on her menstrual period		X
b. Pregnancy	X	
c. Vaginal infection	X	
d. Cardiac disease		X
e. all the above		X
Q25. Regarding the physiology of pregnancy:-		
a. The blood volume increases in pregnancy	X	
b. The plasma volume increases in pregnancy	X	
c. The red cell volume decreases with pregnancy	X	X
d. Normal weight gain in pregnancy is about 28 pounds	X	
e. The liquor amnii increases rapidly for about 35 weeks of pregnancy		
Q26. About the placenta:-		
a. By the 19th day after conception, the placenta is fully active		
b. Chrorionic villi permit gaseous exchange between mother and baby	X	X
c. H.P.L. is manufactured by the placenta	X	X
d. H.C.G. is manufactured by the placenta	X	
e. Placenta arccreta is a normal finding in pregnancy		X

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	TRUE	FALSE
Q27. The average length of the vagina is:-		
a. 6.5 cm		X
b. 7.5 cm		X
c. 9 cm		X
d. 10.5 cm	X	X
e. all the above		X

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Q28. Trial of scar should be stopped immediately if the following occurs:-		
a. Foetal distress	X	
b. Sudden cessation of contractions	X	
c. Tenderness over the scar	X	
d. Sudden tachycardia	X	
e. Poor progress of labour after monitoring for 8hrs	X	

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Q29. Regarding disease of cervix:-		
a. The endocervix has a columnar epithelium	✓	
b. Carcinoma of the cervix starts or develops at the squamo-columnar junction	X	
c. Colposcopy is a means of studying the cervix	X	
d. Cancer of the cervix does not spread to the aortic nodes		
e. A bivalve speculum is used to expose the cervix and its surroundings		X

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Q30. Contra-indication to syntocinon include:-		
a. Placenta Abruption	X	
b. Previous Caeserian Section	X	
c. Cephalo pelvic disproportion	X	
d. Meconium stained liqour	X	
e. Breech presentation	X	

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Q31. Indications to tocolytics i.e. inhibition of labour include the following:-		
a. Severe bleeding	X	X
b. Abruptio placentae	X	X
c. Chorioarunitis		
d. Imminent abortion before 16 weeks of gestation	X	X
e. All of the above	X	X

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Q32. About the disease syphillis:-		
a. It is due to infection by Treponema Pallidum	X	
b. It is an STD	X	
c. Primary chancre is seen early in the disease	X	
d. Penicillin is not effective in the treatment of syphillis		X
e. Lymphogranuloma venereum is common accompanying condition		

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		TRUE	FALSE
Q33.	Diagnostic feature of hydatidiform mole include the following:-		
a.	Protracted nausea	X	
b.	Disproportionate growth of the uterus	X	
c.	Absence of the fetal parts or a boggy consistency of the uterus	X	
d.	The passage of the vesicles	X	
e.	None of the above		X
Q34.	Causes of Disseminated intravacular coagulation include the following:-		
a.	Abruption placenta	X	
b.	Endotoxic shock	X	
c.	Intra uterine cleat fetus syndrome	X	
d.	Eclamptogenic toxemia	X	
e.	all the above	X	
Q35.	Which of the following is/are true about multiple pregnancy:-		
a.	Vomiting and nausea are exagerated	X	
b.	Uterine size bigger than period of amenorrhea	X	
c.	Abortion or premature deliveries are common	X	
d.	Multiple pregnancy is absolute indication for C/S		X
e.	None of the above		X
Q36.	About the normal anatomy in the female:-		
a.	The uterus consists of the cervix and corpus	X	X
b.	The uterus normally lies bent forward at an angle of 90° to the direction of the vagina	X	X
c.	The two ovaries are the homologues of the testis	X	
d.	The labia minora contain no adipose tissue	X	
e.	The uterus has no natural supports		X
Q37.	The following constitute antenatal profile:-		
a.	R.P.R.	X	
b.	Haemogram		X
c.	Liver Function Tests	X	
d.	Blood group and X-matching		X
e.	Blood group and Rhesus Factor		X
Q38.	In Placenta Previa:-		
a.	Type IV meas the placenta completely covers the uterine Fundus	X	X
b.	Digital vagnal examination is indicated during labour		X
c.	Type III mens the placenta ceases to cover the Os as the cervix ilates	X	X
d.	Delivery is alwyas by Caesarean Section	X	X
e.	EUA has no lace in the management pregnancies		X

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		TRUE	FALSE
Q39.	The following are High Risk:-		
a.	History of successful VVF repair	X	
b.	History of laparatomy for intestinal obstruction		X
c.	History of second degree perineal tear		X
d.	Previous congenital abnormality		
e.	Previous two abortions	X	
Q40.	The following are complications of pre-eclampsia :-		
a.	Eclampsia	X	
b.	IUGR	X	
c.	Placenta Obruptio	X	
d.	Stroke	X	
e.	Renal Failure	X	
Q41.	The following are indications for immediate delivery in patients with Pre-eclampsia:-		
a.	Epigastric pain plus blurred vision	X	
b.	Eclampsia	X	
c.	Spotting		X
d.	Deep Venous Thrombosis		X
e.	Impending Renal Failure	X	
Q42.	The following are known complications of Placenta Abruptio:-		
a.	Depletion of Fibrinogen and Platelets	X	
b.	Sheehan's Syndrome	X	
c.	Acute cor pulmonale	X	X
d.	IUFD	X	
e.	Cord prolapse	X	X
Q43.	A male with the following characteristics and/or history is most likely fertile:-		
a.	Oligospermia		X
b.	Testicular volume of 8mls		X
c.	Sperm count of 80 million per ml	X	
d.	History of undescended testis at birth		X
e.	One testicle	X	
Q44.	A patient with an ectopic pregnancy:-		
a.	Always gives history of amenorrhea	X	X
b.	May give history of S.T.D.	X	
c.	The pregnancy may be found in the cervix		
d.	The pregnancy test is always positive	X	X
e.	Is an emergency to be operated on immediately	X	

1101

		TRUE	FALSE
Q45.	Regarding Pap Smear:-		
a.	Is important in Ca-cervix eradication campaign	X	
b.	Class III may be due to cervical infection	X	X
c.	Class III is mainly treated by surgery		X
d.	Class IV means there are malignant cells	X	
e.	The commonest mode of treatment for class Iv is Hysterectomy	X	X
Q46.	Diabetes in pregnancy:-		
a.	May be associated withintra-uterine growth retardation	X	X
b.	May be associated with polyhydramnios	X	
c.	Is better controlled using oral hypoglycaemics		X
d.	Is a disease of those who come from diabetic families only		X
e.	Is associated with foetal abnormalities	X	
Q47.	The following are abnormal findings on semenalysis:-		
a.	40% of spermatozoa not actively mobile at 2 hours after ejaculation		
b.	30% of spermatozoa dead at 2 hours after ejaculation		
c.	Digospermia		
d.	Seminal volume of 0.5 mls	X	
e.	Presence of white blood cells		
Q48.	In Choriocarcinoma:-		
a.	Pregnancy test is negative		X
b.	The disease is sometimes seen without history of pregnancy	X	
c.	Treatment is only by chemotherapy		X
d.	The patient can be treated on out patient basis		X
e.	There is no lung metastasis		X
Q49.	The following are complications of heart disease in pregnancy:-		
a.	Ketoacidotic coma		X
b.	Abruptio placenta		X
c.	Congestive cardiac failure	X	
d.	Deep Venous Thrombosis	X	X
e.	Pleural Effusion		X
Q50.	Vacuum Extraction is indicated in:-		
a.	Foetal Distress	X	X
b.	Cephalopelvic disproportion		X
c.	Maternal Distress	X	
d.	Face Presentation		X
e.	Breech Presentation		X

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$3\frac{1}{2}$

$3\frac{1}{2}$

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$3\frac{1}{2}$

12.5

		TRUE	FALSE
Q51.	About the bony passages of the mother:-		
	a. The pelvis is made up of 4 bones		
	b. The bones are the two innominate bones, the sacrum and the coccyx		
	c. The false pelvis is of no significance		
	d. In a woman whose height is less than 5 feet should have an accurate assessement of the pelvis made over the pregnancy		
	e. Failure of the foetal head to engage at term means a Caesarean Section		
Q52.	a. Antenatal care is largely preventive medicine		
	b. Antenatal care should ensure that the mother reaches the end of pregnancy as healthy as she was at the onset		
	c. Antenatal care should ensure that the mother is delivered of a healthy baby		
	d. Antenatal care provides nutrition education		
	e. Antenatal care should involve only consultant doctors		
Q53.	The following are important in the diagnosis of pregnancy:-		
	a. Amenorrhoea in a woman who has previously menstruated regularly should be considered pregnant		
	b. The uterus enlarges and softens in pregnancy		
	c. Vaginal changes are not specific for pregnancy		
	d. Hegar's Sign may indicate pregnancy		
	e. An Ultrasound Scan is useless in the diagnosis of pregnancy		
Q54.	Regarding the management of labour:-		
	a. Hospital delivery is beneficial		
	b. Traditional birth attendants (TBA's) are dangerous and should be avoided		
	c. False labour does not lead to the birth of the baby		
	d. Partogram was first used in Zimbanbwe in 1972		
	e. Hibitane obstetric cream should never be used in the examination of a labouring woman		
Q55.	About malpositions and malpresentations:-		
	a. Face presentation is a malposition		
	b. Breech presentation is a malpresentation		
	c. Prolapse of the cord is common in transverse lie		
	d. Breech presentation occurs in 3 percent of term labours		
	e. Breech delivery is simple		

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		TRUE	FALSE
Q56.	About hypertensive disease in pregnancy:-		
a.	Pre-eclampsia is common in the very young primigravidae		
b.	Essential hypertension is common amongst multigravidae		
c.	Pre-eclampsia can be mild, moderate, or severe		
d.	Pre-eclampsia is common in the very poor primigravidae		
e.	Magnesium sulphate ($MgSO_4$) is never used in eclampsia treatment		
Q57.	About pain relief in labour:-		
a.	A good doctor will never give powerful drugs unneccessarily		
b.	Early in labour,, tranquilizers are given to reduce anxiety		
c.	Pethidine depresses the respiratory centre		
d.	Epidural anaesthesia is not used much in Kenya		
e.	Paracervical nerve block can not be used instead of the above		
Q58.	Regarding obstructed labour:-		
a.	Cephalo-pelvic disproportion can cause it		
b.	A pelvic tumour can cause it		
c.	Hypotonic inertia occurs in primigravida		
d.	Obstructed labour is rare in developed countries		
e.	Rupture of the uterus does not occur in this condition		
Q59.	Regarding obstetric operations:-		
a.	Forceps are used to deliver the foetal head from the birth canal		
b.	Forceps can also be used to rotate the foetal head		
c.	Forceps are used in anaesthesized patients		
d.	Vacuum extractor can be used to do all the above		
e.	Caesarean Section is never done when the baby is dead		
Q60.	About foetal maturity:-		
a.	A baby born before the 38th week is premature		
b.	Post datism occurs when a baby is born after the 42nd week		
c.	The weight of a baby at term is 2.5kgs and above		
d.	Amniotic creatinine is used to estimate foetal maturity		
e.	Surfactant testing cannot be done in Nakuru		

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